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Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
National Coordinator for Health Information Technology
Department of Health and Human Services
Mary E. Switzer Building
330 C Street SW
Washington, DC 20201

Re: HHS Health Sector AI RFI [RIN 0955-AA13]

Dear Assistant Secretary Keane,

The AAMC[1] welcomes this opportunity to comment on the request for information entitled "**Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care**" 90 FR 60108 (December 23, 2025), issued by the Department of Health and Human Services (HHS) Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP) and Office of the National Coordinator for Health Information Technology (collectively the ASTP/ONC or the Department).

The AAMC commends the Department for its ongoing commitment to leverage technological innovation to enhance patient care delivery. We look forward to ongoing collaboration with the ASTP/ONC to develop a meaningful policy and regulatory framework to advance the adoption and use of artificial intelligence (AI) to improve health outcomes for patients and communities, reduce burden, and drive value in health care while fostering public trust and confidence in technology innovations.

Our comments in response to the request for information follow.

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[1] The AAMC is a nonprofit association dedicated to improving the health of people everywhere through medical education, clinical care, biomedical research, and community collaborations. Its members are all 162 U.S. medical schools accredited by the Liaison Committee on Medical Education; 13 Canadian medical schools accredited by the Committee on Accreditation of Canadian Medical Schools; nearly 500 academic health systems and teaching hospitals, including Department of Veterans Affairs medical centers; and more than 70 academic societies. Through these institutions and organizations, the AAMC leads and serves America's medical schools, academic health systems and teaching hospitals, and the millions of individuals across academic medicine, including more than 210,000 full-time faculty members, 99,000 medical students, 162,000 resident physicians, and 60,000 graduate students and postdoctoral researchers in the biomedical sciences. Through the Alliance of Academic Health Centers International, AAMC membership reaches more than 60 international academic health centers throughout five regional offices across the globe.

EXPERIENCE WITH AI TOOLS IN CLINICAL CARE

The Department seeks feedback on AI tools deployed in clinical care that have met or exceeded performance and cost expectations, as well as where they have fallen short. Academic health systems (AHSs) are at the forefront of innovation in healthcare. In fiscal year 2024, a significant portion of our members reported full integration of certain AI initiatives: 29% for clinical support tools, 25% for predictive analytics for patient care, 23% for AI diagnostics, 10% for population health management, 9% for patient communication and education, and 5% for AI-assisted surgery.[2] As such, our members have experience exploring the use of AI and AI-based tools to improve access to care, streamline administrative tasks, and assist practitioners as they deliver safe, effective, high-quality care. However, at this early stage of use, our members report challenges with measuring the performance of these tools relative to their costs.

Examples of Academic Medicine's Adoption of AI

Administrative Tools

Academic health systems are using AI to streamline the administrative side of patient care. Through the automation of documentation and administrative tasks, AI can help to reduce clinician fatigue and the burden of documentation. These tools may allow clinicians to devote more attention to meaningful interactions with patients during the clinical encounter. For example, ambient listening and scribe technologies help to capture patient-provider conversations and document relevant clinical information in the medical record.
Chart summarization, in-basket message labels, and chatbots that enable plain-language responses to patient queries help to streamline clinical and administrative workflow. AI can provide medical coding and billing assistance by analyzing clinical notes, and EHR data to suggest accurate billing codes. Intake and referral platforms create searchable, actionable data to aid triaging and routing patients for timely care. AI note readers can also extract key metrics across a patient population that health systems can use to address systemic issues in health care. Additionally, AI tools support prior authorization processing, claims denial management, contracting, and operational tasks, including inpatient flow, surgical capacity, and scheduling.

Clinical Support Tools

AI clinical support tools help clinicians to make clinical decisions by analyzing clinical data medical literature, imaging and patient information. Clinical support AI tools are less robust than administrative solutions but are advancing in key areas, including imaging and radiology support, evidence-based clinical decision support, predictive models for patient throughput and discharge planning, streamlined e-consult communication between providers, and remote patient monitoring. These tools do not replace clinician expertise but rather support clinician work and increase efficiency by assisting with diagnosis and treatment decisions.

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[2] Teaching Hospital Characteristics, AAMC analysis of FY2024 American Hospital Association Annual Survey Database. AAMC membership data, December 2025.

Patient Facing Support and Communication

AI tools are able to assist with patient communication managing large volumes of patient messages, drafting responses to routine portal questions (e.g. refills), triaging messages based on clinical topic, and assisting with patient reminders for screening, imaging, and follow-up visits. As well as creating an informational hospital course synthesis for discharge summaries. These tools make it easier for providers to meet patients where they are while increasing meaningful touchpoints.

Medical Education

Training programs are also incorporating AI into medical education. For example, AI-powered simulators for training, adaptive real-time-feedback systems, AI-based analytic models that quantify technical skills and performance dashboards provide longitudinal tracking of skills improvement. These tools allow trainees to practice in a safe, judgement-free environment while enhancing and refining their skill sets before entering a clinical setting.

Concerns with Finding True Return on Investment (ROI) with AI Adoption

Each of the use cases above require human review and interaction – none are truly autonomous. Adoption of AI tools in health care requires significant governance and ongoing monitoring and evaluation over the lifecycle of each tool and use case. AHSs are developing comprehensive governance structures for the adoption of AI tools across their system. Adoption of AI requires heavy initial investment, including legal support to ensure compliance with state and federal law and risk management, reviewing vendor options for purchase, building and customizing tools, infrastructure set-up, integration into existing electronic health record technology, testing and validation of tools, training of clinicians and staff, data management (including privacy and security), continued monitoring, and updates. Given the initial cost and operational effort required to get these initiatives up and running, the return on investment is not always clear. Broad uncertainty around reimbursement increases pressure on organizations to clearly demonstrate ROI to support continued investments. And even if the AI tool can improve access to care, the costs of implementing, upkeeping and maintenance may be prohibitive to long-term use.

BARRIERS TO ADOPTION AND USE IN CLINICAL CARE

High Cost of AI Tools

The market is currently saturated with numerous costly, niche AI-tools. Evaluating these tools is challenging for AHSs given the breadth of options. Implementing multiple AI tools can create a fragmented system that does not work cohesively. As a result, organizations may attempt to create a customized in-house tool, requiring specialized expertise and significant investment to implement and maintain. In many cases, the ROI of AI-enabled tools may not be sufficient to justify the large-scale adoption or long-term utilization of these tools.

Liability Allocation Across All Actors

AI poses new challenges for medical malpractice liability. Traditional negligence frameworks require identification of a party at fault, based on proof that their actions or inactions caused the harm. However, when using AI, this is unlikely to be clear. AI functions in a novel manner and unlike conventional technologies with a clear human creator and/or operator, some forms of AI can generate new ideas, learning and operating dependently after deployment. Our existing liability framework is based on precedent, and case law has not clearly established how the courts would assign liability when working with AI tools. Utilizing AI in clinical care will remain risky without a clear understanding of when providers and health systems assume liability. This risk is amplified by the fact that users of AI tools do not always have access to the data or other information needed to fully understand how the tools function. This is generally because of the black box nature of AI tools and the protection of propriety information. For health systems to assess and plan for the risk associated with using AI, courts or policymakers must address these uncertainties. For example, states could update liability standards, given the fact that AI does not neatly fall into the traditional negligence framework, to distribute fault across actors (AI developers, EHR systems, providers) as is done in aviation when automation fails. Ultimately there must be a clear balance and understanding of risks across actors to accelerate the adoption of AI tools in clinical care delivery.

Patchwork of State Laws and Regulations

Without a regulatory framework at the federal level, states have begun to step in and fill the void to set guardrails for the use of AI broadly and, in some cases, specifically for its use in health care. The result is an increasingly patchwork system with states having a unique set of standards and penalties. It is extremely burdensome for health systems, especially those that operate in different states, to remain in compliance with differing requirements across states and for vendors and developers of AI tools to comply across all states. Health systems will likely consider the compliance demands for adopting AI tools and factor those costs into potential ROI considerations.

Interoperability of Data Systems

Health IT vendor systems often house data in separate repositories in diverse, proprietary formats which may not be able to exchange information effectively. And users are more likely to interpret data differently when presented in different formats. These challenges are compounded by data silos and a lack of transparency around how vendors train and validate algorithms, making it difficult for end users to assess accuracy.

Privacy and Security

As covered entities, AHSs must meet strict requirements for the storing and use of protected health information (PHI) under HIPAA. AI tools possess significant challenges regarding privacy and data security because of the black box nature of the underlying algorithms and coding. It is difficult for health systems to guarantee that PHI has been sufficiently de-identified, in part because as models become increasingly more sophisticated, it is possible to re-identify the PHI. Furthermore, AHSs partnering with non-covered entities or vendors cannot execute control over data once it is available beyond the contained health system. This is problematic because non-covered entities do not have the same fiduciary responsibilities for PHI. Therefore, health systems may prefer to contract with vendors (or develop tools in-house) in a manner to maintain control of the tool's access to PHI, while again balancing the added privacy and security costs with any assessment of the tool's ROI.

REIMBURSEMENT

ASTP/ONC seeks feedback on payment policy changes that ensure payers have the incentive and ability to promote access to high-value AI clinical interventions, foster competition among clinical care AI tool builders, and accelerate access to and affordability of AI tools for clinical care.

Consider a Broad Framework for Establishing Different Payment Pathways for Different Tools

AHSs depend on adequate reimbursement of services provided to spur continued innovation in health care. CMS should ensure market basket updates for both the Inpatient Prospective Payment System (IPPS), Outpatient Prospective Payment System (OPPS), and payment updates for the Physician Fee Schedule (PFS) correct for past insufficient updates and better reflect the cost of providing care to patients. Additionally, payment policies should be updated to establish a framework that supports changes in care patterns driven by AI integration. This may include:

Expanding New Technology Add-on Payment (NTAP) for Hospitals

In 2001, CMS created NTAP, an add on payment for new medical services and technologies used in the inpatient setting. To qualify for NTAP, the technology must be on the market for less than 2-3 years, receive inadequate payment rates, provide substantial clinical improvements, and satisfy certain FDA market authorization requirements. CMS should consider expanding NTAP payments to include the outpatient setting to facilitate the adoption of AI As more services take place in the outpatient setting, NTAP payments could increase investments and utilization of AI to improve care in this setting. CMS should also consider reviewing specific technologies that have been on the market for nearly three years to determine if the timeframe should be extended.

Testing Outcome-Aligned Payments for Clinicians

CMS's Innovation Center recently released a new voluntary alternative payment model that will test outcome-aligned payment approaches to expand traditional Medicare patients' access to technology-supported care to improve health and manage chronic disease. The Advancing Chronic Care with Effective, Scalable Solutions (ACCESS) Model will test payment to Part B-enrolled providers who meet clinical targets for managing their patients' qualifying conditions across four clinical tracks. CMS will evaluate complementary payment incentives to traditional Medicare fee-for-service (FFS) payment to better support novel technology-supported care, like telehealth and Food and Drug Administration-authorized devices or software, that can help people manage chronic conditions with continuous support beyond their doctor's office.

As designed, the ACCESS model contains a broad prohibition from Medicare FFS reimbursement for model participants for patients attributed to the model. This blanket prohibition will dissuade most health care providers from participating in the model, as many Medicare patients need health care services outside of those included within the condition-specific outcome-aligned payments through the model. CMS should consider incorporating the goal to promote novel technology-supported care by testing outcome aligned payments with a more targeted FFS prohibition to allow for greater participation from providers with existing care management relationships with patients. CMS could also consider value-based outcome aligned payments tied to accountable metrics, for example, metrics within value-based purchasing programs that reward acquiring and usage of AI tools that users have proven provide clinical benefits and meaningful ROI.

Expanding as Appropriate Software as a Service Under the Physician Fee Schedule

Software-as-a-Service (SaaS) is algorithm-driven software utilized by clinicians to assist with clinical assessments, including decision support intervention software. Medicare covers SaaS under Part A or Part B that are included in a Medicare benefit category, not statutorily excluded, reasonable and necessary and approved or cleared by the FDA. As appropriate, CMS could consider creating new codes to expand payment for AI-enabled SaaS. However, given the current proliferation of codes and burden associated with valuing each SaaS, we caution the heavy reliance on new codes.

RESEARCH & DEVELOPMENT

ASTP/ONC seeks input on the ways in which the Department may invest in research & development to integrate AI into care delivery.

Research and Implementation Science to Improve Data Hygiene

Data hygiene is integral to ensure that AI tools and algorithms are accurate when supporting clinical care and health care operations. An AI tool's outputs are unreliable and potentially dangerous if the underlying data is flawed or inoperable. This includes data that is incomplete, outdated, mislabeled, duplicative, biased or otherwise inaccurate. As discussed above, data stored in inconsistent formats can increase the risk of differential interpretations and can make it difficult to effectively communicate across different systems. HHS should consider supporting or leading research and implementation science to improve underlying data reliability and interoperability that feeds into AI systems. This could include, for example, standardized quality assurance standards for data collections and utilization, as well as testing algorithmic bias using benchmarks to determine how AI performance fluctuates under different circumstances.

Research to Develop Sounds Frameworks for Ongoing Monitoring and Evaluation

AI systems require ongoing monitoring and evaluation to ensure that they function as intended and meet clinical objectives. One study found that 91% of models degrade over time when conditions shift or change.[3] Significant investment in implementation science, studying how to successfully integrate AI into clinical workflow, is necessary to improve metrics and to foster ongoing trust in AI. This could include assessing outcomes of clinicians who use AI-assisted clinical decision-making tools to determine if clinicians achieve reduced morbidity or mortality rates, costs, or administrative burden. Or developing best practices for ongoing validation that detects issues such as drift detection that alerts users if the AI's performance is diverging from its original training data.

CONCLUSION

Thank you for the opportunity to provide feedback on this critically import topic to inform the Department's future policymaking. We would be happy to work with CMS on any of the issues discussed or other topics that involve the academic medicine community. If you have questions regarding our comments, please feel free to contact Phoebe Ramsey (pramsey@aamc.org) and Ki Rosenstein (krosenstein@aamc.org).

Sincerely,

Jonathan Jaffery, M.D., M.S., M.M.M., F.A.C.P.
Chief Health Care Officer

cc: David Skorton, M.D., AAMC President and Chief Executive Officer

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[3] Vela, D., *et al.* Temporal quality degradation in AI models. *Scientific Reports* (July 2022).